

Adrabetadex Document C: HCPCS and OPPS Pass-Through Application Brief

Prepared for client delivery | March 30, 2026 | Audience: reimbursement, policy, and market access teams

Purpose. This brief covers coding and payment mechanics for launch. It does not restate the clinical value story. It explains how Adrabetadex will move from interim miscellaneous coding to product-specific HCPCS coding and how the hospital outpatient pass-through pathway works under current CMS rules.

HCPCS and interim billing framework

CMS states that miscellaneous codes allow suppliers and providers to begin billing immediately in the absence of a product-specific HCPCS code while a new code request is under review. CMS also states that HCPCS Level II drug and biological codes are updated quarterly and that pass-through coding updates occur quarterly through separate procedures.

The site-of-service distinction matters. CMS's January 2025 Part B drug payment overview states that during the initial sales period when ASP is not yet available, the payment limit is 103% of WAC for separately payable Part B drugs. The same overview states that OPPS drugs without an assigned HCPCS code are paid at 95% of AWP. CMS's hospital outpatient billing article then narrows the bridge-code logic: C9399 is reserved for new FDA-approved drugs or biologics approved on or after January 1, 2004 that do not yet have a specific HCPCS code, while J3490 or J3590 are used for NOC drug or biologic billing when a product no longer qualifies for C9399.

Setting	Interim coding path	Immediate payment frame
Physician office / Part B	Miscellaneous or contractor-priced HCPCS path until a product-specific J-code is active.	103% of WAC during the initial sales period when ASP is not yet available.
Hospital outpatient department	C9399 if the drug is newly FDA-approved and has no specific HCPCS code; otherwise J3490 / J3590 NOC billing as directed by MAC rules.	OPPS and MAC pricing rules apply. CMS states OPPS drugs without an assigned HCPCS code are paid at 95% of AWP.

What the HCPCS application will require

CMS routes HCPCS coding applications through MEARIS. For drug and biologic pass-through items, CMS states that the application package must include the trade and generic name, product description, packaging, dosage form, method of administration, WAC and AWP, ASP if available, actual cost to hospitals, market-availability date, HCPCS code or proposed descriptor, projected utilization by site of care, FDA approval letter, and final label. CMS also notes that a separate application is required for each distinct drug or biologic.

- The filing will need a proposed HCPCS descriptor and billing unit that match how the product will be purchased and administered.
- The package will need hospital outpatient utilization estimates, physician-office utilization estimates, and supporting cost evidence.
- The package will need the final FDA approval letter and the final label before CMS will treat the application as complete.

OPPS pass-through mechanics

CMS's March 2026 OPPS pass-through page states that applications are now accepted only through MEARIS. CMS's current pass-through instructions state that drug and biologic applications are accepted on an ongoing basis, but the completed file must be in by the first business day of March, June, September, or December to target the next quarterly effective date. The same instructions state that transitional pass-through payment is available for at least 2 years but not more than 3 years and is limited to qualifying new drugs, biologicals, and radiopharmaceuticals whose cost is not insignificant in relation to the associated OPPS payment.

Complete application received by first business day of	Earliest pass-through effective date
March	July 1
June	October 1
September	January 1
December	April 1

Adrabetadex-specific coding and timing implications

Using the current expected PDUFA date of August 17, 2026, the earliest realistic OPPS pass-through effective date is January 1, 2027, if the FDA approval letter, final label, utilization file, and pricing file are all complete in time for the first business day of September 2026. If that window is missed, the next quarterly effective date is April 1, 2027.

The physician-office bridge is numerically straightforward. At an illustrative WAC of \$39,000 per 900 mg vial, the Medicare initial-sales benchmark is \$40,170 per administration. At 26 administrations per year, the annual Part B benchmark equals \$1,044,420. That is the most concrete public benchmark available before ASP exists.

The hospital-outpatient bridge is operationally stricter. CMS's outpatient billing article states that hospital claims for new unclassified OPPS drugs must report the NDC, the total quantity administered in the applicable unit of measure, and the date administered. The same article states that incorrect use of C9399 instead of the appropriate NOC code can create a payment error or overpayment. For Adrabetadex, hospital billing education will therefore need to be in place before first commercial dose, not after the first wave of denials.

Adrabetadex coding issue	How the launch file will resolve it
No product-specific HCPCS code on day 1	Use the interim miscellaneous-code pathway allowed by CMS while the permanent or temporary product-specific code request is under review.
Hospital outpatient claim edits	Issue an OPPS billing guide that states when C9399 is valid, when J3490 or J3590 is required, and what NDC and quantity fields must be populated.
Need for separate HOPD support	File the OPPS pass-through package through MEARIS with hospital cost data and projected outpatient volume.
Need for physician-office reimbursement benchmark	Model the initial sales period at 103% of WAC until ASP is published and the product-specific payment file is active.

Sources

- CMS. Overview of Coding & Classification Systems. Updated March 19, 2026.
- CMS. Part B Drug Payment Limits Overview. January 2025.
- CMS. Pass-Through Payment Status and New Technology Ambulatory Payment Classification (OPPS) page. Updated March 19, 2026.

- CMS. Process and Information Required to Determine Eligibility of Drugs, Biologicals, and Radiopharmaceuticals for Transitional Pass-Through Status under OPPS. Updated March 16, 2026.
- CMS. Article A55913: Billing and Coding - Hospital Outpatient Drugs and Biologicals Under OPPS.
- Beren Therapeutics. Internal training deck, January 2026, especially slides 44 and 50.
- Adrabetadex Pre-Approval Information Exchange deck, slide 36.
- Provider Net Cost Model Concept Brief and Launch Access Reality Addendum for Adrabetadex.